

Is this worth doing?

Five tests, a verdict on each, and the conditions I would attach to a yes.

Dr Itunu Akinware, Group Chief Executive, Medbury Medical Services

Itunu, this tests whether to commit NGN 523 M to converting the building into a medical campus, incorporating six companies around it and operating it. It says yes, with three conditions, and all three are on the next page.

Five tests: demand, the building, the money, the funding, and what would break it. Four pass outright. Demand passes on evidence that is real but thin, a consultant survey with 20 responses against a target of 40 to 60.

It returns **41 per cent**, NPV NGN 238 M at a 25 per cent naira hurdle. You need **NGN 523 M** to open and run six months. Peak exposure is NGN 659 M in month 15 and your capital is back in month 33. No terminal value in any of it.

One thing to take from it. **You can overrun the fit-out by 104 per cent and still make money. You can only miss the consultant target by 34 per cent.** The building is not the risk.

Debo Odulana

Founding Partner, Consult for Africa

00 / THE VERDICT

Proceed, on three conditions.

Test	Verdict	On what basis
1 Is there demand?	QUALIFIED PASS	The survey evidences price, frequency and theatre need. It is 20 responses. Widen it before the fit-out is committed
2 Can the building do it?	PASS	6 bookable rooms, 168 prime hours a week, a ceiling of about 50 consultants. Measured, not assumed
3 Does it make money?	PASS	41% IRR, NPV NGN 238 M at 25%, 2.6 times cash back over five years
4 Can it be funded?	PASS	NGN 523 M to open and run six months, peaking at NGN 659 M in month 15
5 What would break it?	QUALIFIED PASS	No single failure kills it. Two demand failures together take the return to single figures

Proceed, subject to three conditions.

One. Widen the consultant survey past 40 responses before the fit-out is committed. It is the thinnest assumption in the pack and the cheapest to firm up. Six weeks and almost no money.

Two. Put a break at year three in the conversion clinic's lease. If consultants sign faster than 50 the ground floor is worth more as bookable rooms than as rent, and a five year term signs that option away.

Three. Start the FCT licence application in week one, alongside the design sprint. It is the only item on the critical path that no amount of money or labour can accelerate later.

01 / IS THERE DEMAND?

Qualified pass. Real evidence, thin sample.

The premium medipark consultant survey ran from 12 to 19 August 2026 and returned 20 responses. The target was 40 to 60, so it is under-powered and should be read with care. What it does say, it says clearly.

	What the survey says	Detail
What they will pay	Optimal price point NGN 65,000, indifference NGN 85,000	van Westendorp, n=16. A 2 hour session at our NGN 35,000 rack rate is NGN 70,000, which sits on the optimal point
How often they would come	1.65 sessions a week on average	Half want two or more a week. Only one respondent wanted none
When they want to come	Evenings and Saturday mornings	Saturday morning 25%, evening 19%, afternoon 19%. Weekday daytime is weakest, which is why our own clinics can share the same rooms

	What the survey says	Detail
Whether they need a theatre	30% do	30% perform procedures needing a full operating theatre, 24% need sedation and an anaesthetist
What they are actually buying	The service, not the room	Nurse or chaperone 85%, address and how it feels 85%, parking and discretion 84%, on-site lab and imaging 75%, billing handled 75%
Where they practise now	64% at a hospital where they have privileges	Only 4% have their own consulting rooms, which is the gap this fills

What the survey broke, and this matters. The membership tiers in earlier work were 1.2M, 2.4M and 4.2M. 25 per cent of respondents said they would not pay a membership at all, and 38 per cent sit under NGN 500,000. Those tiers do not survive contact with this sample and have been replaced with 350k, 750k and 1.5M plus a pay-as-you-go guest rate. A study that only confirms the plan is not a study.

What we do not know

The three gaps

How many consultants exist	The survey tells us what 20 want. It does not size the pool. We need 45 and the ceiling is 50, so the question is whether Abuja holds 50 consultants who want sessional space, not five thousand
How fast they sign	The base case says 45 in 18 months. Nothing in the survey evidences the pace, only the appetite
Whether they pay the rack rate	The optimal price point is a stated preference, not a transaction. Stated willingness to pay usually overstates the real thing

02 / CAN THE BUILDING DO IT?

Pass. Measured, not assumed.

		Basis
Net internal area	664.3 sqm	Extracted from the as-built CAD to about 10 mm. Both floors independently return 70% efficiency, which is the check
Lettable ground floor	177.0 sqm	282.2 less 105.2 of reception and shared services. The theatre takes 64.0, the conversion clinic the remaining 113.0
Bookable rooms	6	3 consulting and 3 procedure. The infusion suite is dedicated and leaves the pool
Prime capacity	168 hours a week	Weekday 17:00 to 21:00 plus Saturday, across six rooms
Consultant ceiling	50	At 1.65 sessions a week of 2 hours each. We plan for 45, just under it
Our own clinics	3,693 room hours a year	They run in the working day, when the consultants do not want the rooms

The two things that are not in our gift

Risk	Lead time	Why it matters
Long lead items	6 to 12 weeks	Cooling, medical gas, nurse call. They must be ordered in week one against the design, not when the contractor mobilises. Ordered late, no amount of labour recovers a month 3 opening
Regulatory licensing	Unknown	FCT facility registration, fire safety certificate, environmental and clinical waste permit. You cannot open a clinical facility without them. This is the real gate on the opening date and it is the one item money cannot compress

03 / DOES IT MAKE MONEY?

Pass, and not marginally.

	NGN M a year	Note
Rent, downstairs	68.3	Diagnostics and pharmacy from month 3, the conversion clinic from month 6. Contracted, so no demand risk
Sessional, upstairs	369.5	Room fees, equipment, membership
Theatre	151.8	Own lists plus external surgeons
Our own clinics	1,008.0	Aesthetics, hair, bariatric medical, infusion
Group revenue	1,597.7	
Group EBITDA	459.7	29% margin, after our management fee
Attributable to Medbury	399.7	After partner equity in the clinics
Returns		Note
Internal rate of return	41%	Five years, nominal naira
Net present value at 25%	NGN 238 M	No terminal value, so a floor
Cash returned per naira in	2.6 times	Over the peak ever invested
Stops burning cash	Month 10	8 months after opening
Capital returned	Month 33	31 months after opening

04 / CAN IT BE FUNDED?

Pass. And the cheque is smaller than the peak.

	NGN M	Note
Category A fit-out, after the line by line review	237.2	Two months on site
Clinical and basic equipment, the six let rooms	36.3	Couches, procedure lights, monitors, autoclave, resus trolley, front office
Launch marketing	30.0	Brand, website and booking front end, launch and consultant open evenings
Setup programme fee	45.5	NGN 25 M of it on mobilisation
Pre-opening operating cost	15.7	Months 1 to 2
Operating cost, first six trading months	108.3	Management fee included
Capital for the four clinics	149.0	Equipment, opening stock, working capital
Less revenue, first six trading months	-38.5	
Less contributions on grant	-61.1	Occupier cash on grant of each sub lease
Minimum to open and run six months	522.5	Building only, without the clinics: NGN 373.5 M

The NGN 115 M already committed is spent and is not a new cheque. Peak exposure is NGN 659 M in month 15, which is deeper than the opening cheque because the theatre capital lands in months 11 to 13 while the consultant ramp is still young. If the theatre slips, the trough is shallower rather than deeper.

05 / WHAT WOULD BREAK IT?

Qualified pass. One failure survivable, two not.

Case	Peak	IRR	NPV	Payback
Base case	659	41%	238	31
Only 30 consultants, taking 30 months	737	21%	-60	40
Survey optimistic: 1.2 sessions a week at 28k	703	25%	6	37
No theatre, ever	647	36%	129	30
Alameda walks, no ground floor tenant	691	34%	137	33
Own clinics 30 per cent below plan	695	29%	52	36
Fit-out 25 per cent over, three months late	770	33%	129	34
Two demand failures together	750	11%	-189	47
Three together, with no theatre	656	1%	-263	56

How far each number can fall before the value goes

Driver	Base	Breaks at	Room to move
Consultants signed	45	30	34% below plan
Rack rate an hour	35,000	21,138	40% below plan
Sessions per consultant a week	1.65	0.91	45% below plan
Months to reach the target	18	46	28 months of slack
Category A fit-out	237.2	483.4	104% over budget

The asymmetry is the finding. Losing Alameda entirely still returns 34 per cent, because the ground floor rent is a small part of a NGN 1,598 M business. Losing the theatre is survivable. A 25 per cent fit-out overrun with a three month delay is survivable. But 30 consultants arriving slowly, combined with the survey being optimistic on price and frequency, takes the return to single figures, and adding a third failure means it never pays back at all.

So the money is not at risk from the thing everyone will spend the next three months discussing. **It is at risk from a recruitment campaign nobody has run yet, resting on a survey of 20 people.** That is where the attention and the contingency belong.

06 / RISK REGISTER

What we would do about each one.

Risk	Likelihood	Impact	What we do about it
Consultants sign slower than planned	High	Severe	BD officer in post from month 1 and the recruitment line spent. Pre-sell founding memberships before opening. Widen the survey now
The survey overstates willingness to pay	Medium	Severe	Guest rate at rack with no commitment, so the first booking is a low barrier
Licence delays the opening	Medium	High	Apply in week 1 alongside the design sprint, not at practical completion
Long lead items arrive late	Medium	High	Ordered in week 1 against the design. A sequencing decision, not a budget one
Alameda does not sign	Medium	Low	The campus still returns 34 per cent. Let the ground floor sessionally instead
No surgical anchor for the theatre	Medium	Medium	Theatre capital is phase two and gated. Bariatrics launches medical first
Fit-out overruns	Medium	Low	104 per cent of headroom before it matters, and nothing clinical is deferred
Related party pricing challenged	Low	Medium	Transfer pricing file built alongside the sub leases in month 1
Returns	Note		
Internal rate of return	41%	Five years, nominal naira	

Returns		Note
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Areas measured from the as-built CAD to about 10 mm; the chalet and boys' quarters have no as-built drawing and remain estimates. Demand evidence from the premium medipark consultant survey, n=20, 12 to 19 August 2026, against a target of 40 to 60. The fit-out is an AACE class 4 estimate, minus 15 to plus 30 per cent, before a tendered bill of quantities. Aesthetics volume, membership uptake and the bariatric case rate are the least evidenced assumptions. Related party charges are subject to a transfer pricing opinion. Not a binding offer, and not legal or tax advice.